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PSYCHOLOGICAL ASSESSMENT REPORT

SOCIODEMOGRAPHIC DATA

NAME: Ms. Apoorva Sharma

AGE: 26 years

GENDER: Female

DATE OF BIRTH: 01/09/2000

EDUCATION: Graduated

OCCUPATION: Home maker

MARITAL STATUS: Married

RELIGION: Hindu

SOCIO-ECONOMIC STATUS: Middle Socio-economic strata

OPD. NO.: 251104003

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TEST ADMINISTERED ON: 27/11/2026

AADHAR NO.: 430147210813

INFORMANTS DETAILS:

NAME

RELATIONSHIP

DURATION

1. Rahul Sharma

Husband

4 Years

The information provided by the informant was reliable and adequate.

REASON FOR TESTING: For psychological assessment to evaluate nature and severity, and for treatment plan.

CASE WORKED UP BY: Ms. Abhilasha Wanchoo (M.Phil. Second Year Trainee)

Mr. Tejas Dangodra (M.Phil. First Year Trainee)

SUPERVISED BY: Ms. Samridhi Oli (Assistant Professor & Clinical Psychologist MAN college)



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CHIEF COMPLAINTS

1. Impulsive behavior since 6 years
 2. Marital conflict
 3. Irritability and frequent anger outbursts.
 4. Self-harming behaviour
 5. Increased overthinking
 6. Disturbed sleep since 2–4 months.
- } 3 years
- } 2.5

BRIEF HISTORY

The client is a 26-year-old married woman from a middle-class background who presented with emotional and behavioral difficulties that have been affecting her personal and marital life.

She experienced early parental loss, following which her living situation changed, leading to emotional stress and feelings of insecurity. During her young adulthood, she experienced social isolation, which caused distress, loss of independence, and emotional vulnerability due to her household restrictions. Since 6 years as per her sister she has exhibited impulsive behavior on numerous occasions.

She got married 4 years back; however, the marital relationship has been marked by ongoing conflicts and emotional dissatisfaction which leads to frequent overthinking and inability to manage her emotions. These marriage-related problems have contributed to increase irritability and anger outbursts where she harmed herself in presence of her husband as a way to express emotional pain and seek emotional support. Although the frequency has reduced significantly, and her husband reports that she has excessive need for reassurance.

She reports increased overthinking in relation to bearing a child and about her relationship with her husband and his family. Due to her overthinking she often feels upset and experiences distress. As per the patient she is also experiencing disturbed sleep and to initiate sleep she has to take medications. Due to persistent complaints of disturbed sleep she occasionally consumes the prescribed medications in extra dosage to fall asleep.



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TESTS ADMINISTERED

1. Rorschach Inkblot Test
2. Thematic Apperception Test (TAT)
3. Millon Clinical Multiaxial Inventory-III (MCMI-III)

RORSCHACH INKBLLOT TEST

RATIONALE OF THE TEST

The Rorschach Inkblot Test is included to evaluate a person's thought processes, emotional state, and personality traits. As a projective test, it helps uncover unconscious thoughts, coping styles, and interpersonal patterns that may not be easily expressed through direct conversation. It assesses how a person perceives and interprets their environment, providing insight into reality testing, emotional control, and problem-solving abilities. The test is useful in identifying mental health concerns, personality traits, and stress management styles.

Rorschach Interpretive Summary:

Rorschach findings on the above 9 domains are as follows:

1. Control and Stress Tolerance

The present Rorschach findings suggest that the individual possesses a generally adequate capacity for control and stress tolerance, with no marked vulnerability to situational stress or disorganization under routine demands. This is consistent with the clinical observation that she is able to function relatively well when environmental demands are predictable or when she is emotionally distanced from salient interpersonal figures, such as when she is not in close proximity to her husband.

However, the somewhat limited coping resources, indicate the presence of unresolved internal tensions and unmet emotional needs that



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intrude into ongoing thought processes. These internal pressures are likely to become more prominent in emotionally charged interpersonal contexts, particularly those involving perceived attachment threats.

2. Situationally Related Stress

The situational stress is not a salient feature in the present, and interpretations related to stress-induced strain or disorganization are not supported.

3. Affect

Her affective functioning reflects an ambivalent processing style, marked by inconsistency in the degree to which emotions influence thinking and behaviour. While she does not demonstrate pervasive affective dysregulation, the limited modulation of emotional expression suggests that feelings, once activated, are likely to be expressed directly and with intensity.

Clinically, this aligns with reports of anger outbursts and heightened emotional reactivity in interactions with her husband and in-laws, especially in situations where attention, reassurance, or validation is perceived as insufficient. The history of marital infidelity appears to serve as a significant interpersonal trigger, amplifying fears of neglect or abandonment and intensifying demands for attention and proximity when the husband is present.

4. Information Processing

Information-processing findings indicate generally adequate engagement and organization, though the tendency toward global goal orientation and an underincorporative scanning style suggest that she may strive for broad or ideal outcomes without consistently attending to situational constraints or relational nuances. This pattern may contribute to frustration and conflict when expectations of exclusive attention or responsiveness are not met, particularly within complex family dynamics involving in-laws.

5. Cognitive Mediation

Cognitive mediation remains largely intact, with no evidence of enduring reality-testing impairment. Deviations observed appear situational rather than structural, supporting the



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impression that her difficulties are not rooted in psychotic or severe cognitive dysfunction, but rather in emotionally driven interpretations of interpersonal situations.

6. Ideation

The individual shows an ambivalent decision-making style, characterized by inconsistency in thinking and problem-solving approaches. At times, decisions are made through controlled, deliberative thinking, while at other times they are more intuitive and emotionally influenced, which may reduce efficiency and increase vulnerability to judgment errors.

It is suggested that ideational sets and values are well fixed and relatively inflexible, making changes in attitudes or perspectives difficult. Additionally, a high level of internally driven mental activity, which is likely to be chronic and may interfere with attention and concentration during everyday functioning.

7. Self Perception

Self-perception findings point to an average level of self-focus, with increased concern regarding self-image and subjective self-evaluation. This is likely reflected behaviourally in heightened sensitivity to perceived slights and a strong need for affirmation from significant others.

Interpersonally, while overall interest in others appears somewhat limited, perceptions of people are reality-based, and there is evidence of an underlying expectation of cooperation and positive engagement. The imbalance between adaptive and less adaptive human responses suggests that interpersonal functioning may fluctuate depending on emotional state, perceived security in relationships, and the availability of reassurance.

8. Interpersonal Perception and Behaviour

The sum of human content falls below the expected range, suggesting relatively limited interpersonal interest. With Pure H constituting less than half of total human responses in a record of adequate length, the findings indicate reduced engagement with others, although perceptions of people remain reality-based.



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It is suggested that interpersonal behaviors may at times be less adaptive or effective than desirable. At the same time, the presence of cooperative responses and low aggression indicates that the individual generally anticipates positive interpersonal exchanges and shows an underlying interest in participation. The specific quality of interpersonal functioning is therefore likely to be influenced by other factors such as coping style and self-image, rather than by a lack of social intent or capacity.

Overall Impression

The findings suggest adequate control, stress tolerance, and intact reality testing under routine conditions. However, limited coping resources and unmet emotional needs become prominent in emotionally significant interpersonal contexts, leading to intense affective expression, anger outbursts, and black-and-white evaluations. Information-processing patterns indicate high expectations with limited situational flexibility, contributing to frustration and interpersonal conflict, particularly within close family and marital relationships. Overall, the difficulties are context-dependent and relationally triggered, rather than reflective of global emotional or cognitive dysfunction.

THEMATIC APPERCEPTION TEST (TAT)

RATIONALE OF THE TEST

The Thematic Apperception Test (TAT) was administered to explore the client's deeper emotional dynamics, unconscious conflicts, and interpersonal patterns that may not be easily expressed through direct questioning. By creating stories in response to ambiguous pictures, she projects her internal thoughts, feelings, and relational themes, allowing access to underlying motivations and unresolved emotional concerns. The TAT helps clarify how she perceives herself, others, and stressful situations, and reveals patterns such as emotional sensitivity, interpersonal ambivalence, or withdrawal tendencies. This projective method provides insight into her coping style, dominant affective themes, and internal struggles. The results support a more comprehensive understanding of her psychological functioning and guide further clinical intervention.



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Integrated Summary

The analysis of the subject's TAT responses indicates that she was able to comprehend the task and generate coherent stories with a clear beginning, progression, and outcome. The narratives were adequately structured and meaningful, suggesting adequate comprehension and average general intellectual functioning. The stories were largely realistic and emotionally grounded, with strong personal involvement, indicating high self-referential thinking.

Across the cards, the heroes were frequently female characters closely resembling the subject herself, reflecting strong identification. The central figures were consistently portrayed as emotionally sensitive, burdened by interpersonal stress, and struggling with feelings of sadness, anger, and helplessness. A recurring emotional tone across the stories was emotional distress combined with internal conflict, particularly in the context of close relationships.

A prominent pattern across the narratives was the dominance of need for intraggression, where emotional pain, anger, guilt, and disappointment were largely directed inward rather than expressed outwardly. The subject repeatedly depicted characters who blamed themselves, questioned their own worth, or suppressed anger to maintain relationships. This was evident in stories involving marital conflict, betrayal, emotional neglect, and perceived injustice, where the protagonist internalized responsibility even when external factors were prominent.

Strong need for Affiliation and need for Nurturance needs were also evident, particularly in stories involving maternal roles and caregiving relationships. The subject demonstrated a deep concern for emotional connection, security, and responsibility toward others, especially children. However, these affiliative needs frequently appeared unmet, contributing to feelings of rejection, emotional deprivation, and vulnerability. The desire for closeness often coexisted with fear of abandonment and disappointment.

Interpersonal relationships, especially with significant others such as the husband and family members, were commonly depicted as emotionally unsupportive, distant, or invalidating. Themes of infidelity, exclusion, authority, and lack of emotional understanding recurred across multiple cards. Male figures were often perceived as dominant, emotionally detached, or self-focused, while



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the female protagonist appeared compliant, restrained, or silently distressed. This reflects a relational pattern marked by power imbalance, emotional dependency, and difficulty asserting personal needs.

Environmental press across the stories was largely interpersonal rather than situational, characterized by criticism, rejection, emotional neglect, and role-based expectations. While overt hostility was rarely depicted, the environment was frequently experienced as emotionally unsafe or invalidating. In contrast, non-human environments, particularly nature and solitude, were consistently perceived as supportive and restorative, indicating reliance on withdrawal and isolation as a coping strategy.

In terms of coping, the subject demonstrated a tendency toward passive and avoidant coping mechanisms, including withdrawal, emotional suppression, and self-soothing through solitude. While there were moments of adaptive coping such as nurturing others, engaging in calming activities, or seeking emotional distance to regulate affect, most story endings lacked clear resolution. This suggests difficulty in actively processing emotional pain or achieving constructive closure.

Overall, the TAT profile reflects significant unresolved emotional conflicts, with prominent themes of loss, rejection, guilt, anger turned inward, and unmet emotional needs. Despite emotional vulnerability, the subject shows adequate ego strength, reality orientation, and moral awareness. However, her coping style appears constrained by emotional inhibition, self-blame, and fear of confrontation. The narratives suggest a strong longing for emotional safety, validation, and understanding, coupled with difficulty trusting others to meet these needs.



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MILLON CLINICAL MULTIAXIAL INVENTORY-III (MCMI-III)

RATIONALE OF THE TEST

The MCMI-III assesses personality characteristics and current clinical symptoms. The results provide insight into the client's personality style, emotional functioning, and areas of psychological distress. Additionally, the test was administered to corroborate and confirm the emerging clinical profile obtained from behavioural observations, interview data, and other assessment measures. Using the MCMI-III helped ensure that the personality patterns identified were consistent, reliable, and not solely based on situational factors or self-report biases.

TEST RESULTS AND INTERPRETATIONS

MODIFYING INDICES (Validity)

The client's validity profile suggests a highly interpretable yet notably distressed response style. The **Disclosure scale** is markedly elevated, indicating a high level of self-disclosure and a pronounced willingness to acknowledge psychological difficulties. This is paired with a very low **Desirability score**, confirming no attempt to present herself in an overly favorable or socially desirable manner. The **Debasement scale** is significantly elevated; this may reflect a tendency to endorse items in a way that communicates negative self-evaluation, emotional pain, and feelings of worthlessness. Taken together, these indices indicate that she is openly communicating with her psychological suffering and is currently experiencing substantial emotional turmoil, which is also corroborating with her history. Although the elevations may suggest that her negative self-view is intensifying the perceived severity of her difficulties, the profile is still considered a valid reflection of her current distress.

CLINICAL PERSONALITY PATTERNS

<i>Scale</i>	<i>BR Score</i>	<i>Clinical Significance</i>
<i>Schizoid</i>	<i>87</i>	<i>Pathological level</i>
<i>Depressive</i>	<i>83</i>	<i>Trait Level</i>
<i>Negativistic</i>	<i>101</i>	<i>Pathological level</i>



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The patient's profile shows a marked elevation on the **Negativistic (Passive-Aggressive)** scale, which suggests that she may display a longstanding pattern of internal conflict, inconsistent responsiveness, and difficulty sustaining cooperative interactions. This elevation is often associated with ambivalence in relationships, where she may fluctuate between agreement and withdrawal. At this level, negativistic features may form a meaningful part of her personality functioning and may influence how she manages interpersonal expectations and emotional expression.

A secondary elevation is observed on the **Schizoid scale**, which is generally associated with emotional distancing, limited affect, and reduced social engagement. This pattern is commonly associated with reduced social engagement, experience discomfort with emotional closeness, or show a restricted range of interpersonal expression. When considered alongside her negativistic traits, this combination is often associated with interpersonal fatigue or withdrawal or inconsistent for her, leading to periods of withdrawal or reduced engagement.

The **Depressive scale** is also significantly elevated. This profile is commonly associated with persistent negative self-evaluation, lowered mood, and a generally pessimistic outlook. For her, this may manifest as diminished motivation, reduced self-confidence, and a tendency to expect limited positive outcomes in interpersonal or emotional situations. In combination with her other personality elevations, depressive tendencies may contribute to ongoing internal dissatisfaction and reduced psychological resilience.

Overall, these interacting elevations suggest a personality pattern characterized by emotional reserve, relational ambivalence, and a tendency toward self-critical or pessimistic thinking. The presence of negativistic, schizoid, and depressive features may contribute to challenges she faces in maintaining consistent interpersonal engagement and regulating emotional responses. These patterns provide a useful framework for understanding aspects of her current emotional experience and interpersonal functioning.



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SEVERE PERSONALITY PATHOLOGY

Scale	BR Score	Clinical Significance
Schizotypal	91	Pathological level
Borderline	89	Pathological level
Paranoid	92	Pathological level

The elevation on the Paranoid scale suggests that she may display a heightened level of vigilance and sensitivity in interpersonal situations. This pattern is often associated with a cautious, guarded, and protective interpersonal stance. For her, this may result in approaching interactions with increased alertness to possible criticism or misunderstanding. These elevations point toward a possible pattern characterized by a style in which trust may develop more slowly and with greater difficulty.

The elevation on the Schizotypal dimension indicates that she may experience cognitive-perceptual sensitivity and interpersonal discomfort. This can involve moments when she has difficulty interpreting social cues clearly, becomes internally preoccupied, or prefers emotional distance in situations that feel overwhelming. For her, these tendencies may lead to social hesitation or withdrawal, particularly when environments feel unpredictable or emotionally demanding. These features suggest a style that may become more cautious and introspective during periods of stress.

Her borderline elevation reflects significant variability in mood, self-perception, and coping stability. She may experience rapid emotional shifts, heightened reactivity to interpersonal stress, and difficulty maintaining consistent emotional regulation. These fluctuations can make it challenging for her to sustain steady responses during relational tension or emotional ambiguity. This profile further suggests that her sense of identity may shift according to emotional or situational factors, contributing to periods of internal conflict and uncertainty.

Taken together, these elevations suggest a complex personality configuration marked by interpersonal sensitivity, cognitive-perceptual uncertainty, and emotional variability, where the



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interaction among paranoid, schizotypal, and borderline features indicates that she may have difficulty maintaining stable interpersonal relationships and emotional balance, particularly during stress. These tendencies can sometimes contribute to increased internal preoccupation, reduced social comfort, and challenges in sustaining consistent regulation of thoughts and emotions, providing a clinically useful framework for understanding the areas in which she may benefit from structured emotional support, stabilization strategies, and therapeutic consistency. When considered together, these elevations may reflect traits that are often observed in individuals with borderline-related patterns of functioning, which often include emotional sensitivity, fluctuating interpersonal engagement, and a variable sense of self. In her case, paranoid and schizotypal features may emerge when she is emotionally overwhelmed, contributing to perceptual uncertainty and interpersonal caution, while her negativistic and depressive tendencies may influence how she experiences relationships—leading to ambivalence, withdrawal, and self-critical thinking. This interconnected pattern suggests that her severe personality elevations are not separate issues but rather reflect multiple facets of a unified borderline vulnerability, helping explain her difficulties with emotional regulation, maintaining stable relationships, and managing perceived interpersonal threats or inconsistencies.

GROSSMAN PERSONALITY FACETS

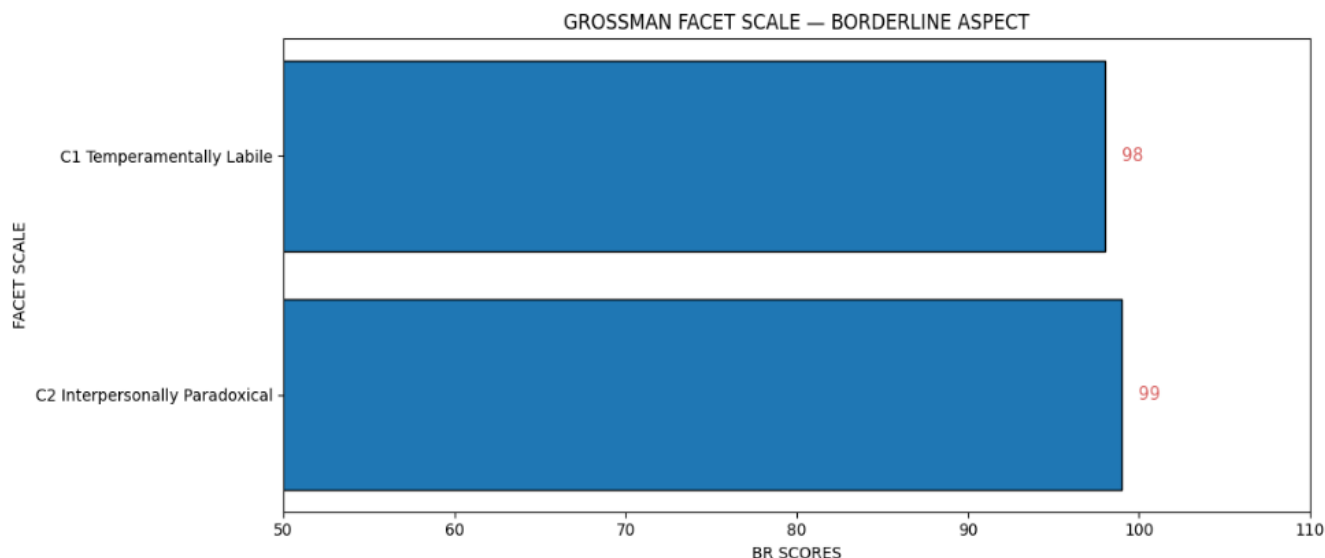
The Grossman facet scales are meant to help explain why certain personality scales are high on the Clinical Personality Patterns and Severe Personality Pathology scales. They show which exact parts of personality functioning (e.g., self-image, interpersonal relations) are contributing to those high scores. By carefully looking at the facet scale results, we can identify which personality traits stand out the most for her.

C Borderline – The domains within this severe personality pattern include C.1 Temperamentally labile, C.2 Interpersonally paradoxical, C.3 Uncertain Self-image.



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C1 Temperamentally Labile:

She obtained a raw score of 9, which corresponds to a BR score of 98, indicating a strong tendency toward rapid and intense emotional shifts. She may frequently experience sudden changes in mood such as going from calm to upset or from affectionate to angry within short periods. This pattern suggests emotional volatility, heightened sensitivity, and difficulty maintaining stable affective states, especially under stress or interpersonal tension.

C2 Interpersonally Paradoxical:

She obtained a raw score of 8, which corresponds to a BR score of 99, indicating a strong pattern of contradictory interpersonal behavior. She may approach others with a desire for closeness but then push them away through anger, mistrust, or impulsive reactions. This push-pull relational style can confuse others and create unstable, intense relationships where she alternates between seeking support and rejecting it.



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CLINICAL SYNDROMES

<i>Scale</i>	<i>BR Score</i>	<i>Clinical Significance</i>
<i>Anxiety</i>	<i>83</i>	<i>Trait Level</i>
<i>Somatoform</i>	<i>107</i>	<i>Pathological level</i>
<i>Bipolar (Manic)</i>	<i>87</i>	<i>Pathological level</i>
<i>Dysthymia</i>	<i>83</i>	<i>Trait Level</i>

The significant clinical elevation on the **Somatoform scale** suggests a significant level of somatic focus. At this intensity, she is likely to experience psychological distress primarily in physical terms, often interpreting normal bodily sensations as medically concerning. This pattern does not confirm a somatic disorder but indicates that emotional tension may be expressed somatically. Such amplification can contribute to frequent worry about health, increased monitoring of physical sensations, and difficulty distinguishing emotional discomfort from bodily symptoms. This cycle may heighten anxiety and functional disruption, as physical sensations reinforce emotional distress.

Clinically relevant elevations are also present on the **Bipolar–Manic** and **Anxiety** scales. The Bipolar–Manic elevation suggests periods of increased emotional activation, irritability, or heightened energy. These fluctuations do not indicate full mania but may contribute to instability in mood and interpersonal functioning. In combination with personality tendencies noted on the Borderline and Paranoid scales, this elevation increases the likelihood of rapid emotional shifts and difficulty regulating reactions during stress.

The Anxiety elevation reflects persistent physiological tension, excessive worry, and sensitivity to interpersonal stressors. This may contribute to restlessness, difficulty relaxing, and heightened alertness to potential threats. Anxiety may interact with her somatic focus, increasing attention to bodily sensations and reinforcing anxious interpretations.



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Additional elevations on **Dysthymia** score suggest a longstanding pattern of low mood, fatigue, and reduced optimism. This chronic depressive tone may lower resilience and increase feelings of inadequacy.

SEVERE CLINICAL SYNDROMES

Scale	BR Score	Clinical Significance
<i>Major Depression</i>	<i>104</i>	<i>Clinically Significant</i>

The significant clinical elevation on the **Major Depression scale** (BR = 104) is consistent with a pattern associated with severe depressive features marked by pervasive sadness, hopelessness, and impaired motivation. At this level, she is likely experiencing significant difficulty in daily functioning, with possible disruptions in sleep, appetite, concentration, and emotional regulation which also aligns with what she has reported during the sessions. This presentation appears consistent with the chronic depressive tendencies reflected in her Dysthymia and Depressive personality scale elevations, suggesting a longstanding pattern that intensifies under stress. These patterns reinforce each other, with negative self-beliefs sustaining low mood and low mood reinforcing negative perceptions of herself and her circumstances.



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CLINICAL IMPRESSION

The overall assessment profile indicates that the client is experiencing significant emotional distress, marked mood instability, and longstanding personality-based vulnerabilities that affect her perception of herself and her relationships. The validity indices confirm that she responded openly and consistently, with elevated distress influencing her self-evaluation but not invalidating the results.

Her personality pattern is characterized by a combination of negativistic, schizoid, and depressive traits. This configuration suggests internal ambivalence, emotional withdrawal, and persistent self-critical thinking. These tendencies appear to contribute to fluctuating engagement in relationships, reduced emotional expression, and difficulty sustaining cooperative or consistent interpersonal interactions.

Additional elevations on paranoid and schizotypal dimensions indicate heightened interpersonal sensitivity, cognitive-perceptual uncertainty, and discomfort in emotionally demanding or unpredictable social environments. These features may intensify during stress, resulting in increased caution, mistrust, and internal preoccupation.

Borderline personality features are strongly indicated, especially in the domains of emotional lability and paradoxical interpersonal behavior. The Grossman facet elevations support this pattern, showing rapid mood shifts and contradictory relational tendencies, such as seeking closeness while simultaneously withdrawing or reacting intensely. Her sense of self shows comparatively less disturbance but may fluctuate under stress.

Clinically, there is notable evidence of somatic preoccupation, with distress frequently expressed through physical symptoms. This somatic focus interacts with her anxiety, contributing to heightened monitoring of bodily sensations and increased worry about health. Elevated bipolar–manic and anxiety scores indicate periods of heightened arousal, irritability, and restlessness, which further challenge her capacity for emotional regulation.



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There are also significant depressive features, including substantial elevations in both dysthymic and major depressive domains. This pattern suggests a chronic and pervasive low mood that intensifies during periods of stress, leading to hopelessness, reduced motivation, and impaired functioning. The depressive and personality factors appear to reinforce each other, contributing to a cycle of negative self-appraisal and emotional instability.

Overall, the clinical picture suggests a complex interplay of mood dysregulation, interpersonal sensitivity, somatic focus, and chronic depressive tendencies within a borderline-structured personality pattern. These combined features contribute to difficulties in emotional stability, interpersonal functioning, self-regulation, and stress tolerance. Continued therapeutic support, emotional stabilization, and consistent treatment structure are recommended to address these interrelated challenges.

OVERALL IMPRESSION

The overall assessment indicates that the client has intact reality testing, adequate basic cognitive functioning, and the ability to manage routine demands. However, she shows significant emotional vulnerability that is significant in close interpersonal relationships. Across projective and objective tests, a consistent pattern emerges of intense emotional sensitivity, unmet affiliation needs, fear of abandonment, and difficulty regulating affect in emotionally demanding situations.

The findings suggest emotional sensitivity and vulnerability, particularly in the context of close relationships. She tends to experience distress internally, which may at times be expressed through physical concerns. Emotional difficulties appear to be situational and relationship-related rather than affecting overall functioning.



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RECOMMENDATIONS

- Long-term psychotherapy is strongly recommended, with a focus on emotional regulation, interpersonal effectiveness, and identity stabilization. A structured approach such as **Dialectical Behavior Therapy (DBT)** or an integrative trauma-informed therapy would be appropriate.
- **Couple Therapy:** Since there are frequent conflicts in the marriage, couple therapy sessions can be useful in addressing misunderstandings, improving communication, and rebuilding emotional connection.
- **Psychoeducation:** Educate the patient and her husband about emotional regulation, communication styles, and stress management. Helping them understand her emotional responses can reduce blame and improve family support.
- **Follow-Up:** Schedule regular follow-up sessions to monitor her progress and adjust the treatment plan as needed.

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